UNDER THE SISTER HOOD

The Unspoken Curriculum

How Women Navigate a Fragmented System and Forge New Paths to Wellness Through Advocacy and Community

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EXECUTIVE SUMMARY

- Across 45 participants from diverse backgrounds and countries, a clear pattern emerged of women reframing medical dismissal not as a personal flaw, but as a failure of a fragmented healthcare system.
- This systemic breakdown, compounded by cultural silence around women's health, creates profound isolation and shame during key life transitions like postpartum and menopause.
- In response, women are cultivating powerful self-advocacy, learning to trust their own bodies and demand the care they deserve.
- Ultimately, the most potent protective factor is community, where the act of sharing personal stories serves as a lifeline for validation, connection, and collective healing.

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PARTICIPANT DEMOGRAPHICS

45	24–65	17	30
PARTICIPANTS	AGE RANGE	ETHNIC BACKGROUNDS	REGIONS

AGE DISTRIBUTION

18-24	1
25-34	6
35-44	22
45-54	9
55-64	6
65+	1

Mean: 43.1 | Median: 43

ETHNICITY

Caucasian	19 (42.2%)
WOC	8 (17.8%)
Indian	3 (6.7%)
Mexican	2 (4.4%)
South Africa	1 (2.2%)
Canadian	1 (2.2%)
Japanese	1 (2.2%)
Carribean	1 (2.2%)
Israeli	1 (2.2%)
Afghani	1 (2.2%)
Filipino	1 (2.2%)
Latino	1 (2.2%)
Australian	1 (2.2%)
Trinidadian	1 (2.2%)
Pakistani	1 (2.2%)
Chinese	1 (2.2%)
Nigerian	1 (2.2%)

GEOGRAPHIC SCOPE

33	12
US-Based	International

PA	4
CT	3
CA	3
MA	3
Canada	2
NY	2
AZ	2
NJ	2

+ 22 more

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KEY INSIGHTS

When faced with a healthcare system that dismisses their symptoms or labels them as emotional, women are finding empowerment by reframing the experience not as a personal failing (e.g., being 'crazy' or 'dramatic'), but as evidence of a broken system, which shifts the focus from self-blame to seeking validation and advocating for change.

> *"Your moods and your hormonal chaos is not a character flaw. It's a biological sign. So first of all, I always say this, you're not crazy, you're not dramatic, and you're not overreacting, right? You are just overdue for supports and overdue to be seen as to be understood."*
>
> **— PATIENT**
> EP. UTS_S4_PODCAST_042.DOCX

Significant gaps in medical education and training, particularly around hormonal health, menopause, and survivorship care, result in fragmented and inadequate support for women. Patients find their symptoms are often misunderstood or misattributed, such as hormonal issues being treated solely as mental health conditions, forcing them to navigate a system that lacks a holistic understanding of their health.

> *"I was putting together puzzle pieces, but not just puzzle pieces. It was almost like someone had dumped a box of puzzle pieces. On me. I didn't have the top of what that puzzle looked like, and I didn't know what was survivorship, what was menopause, what was just, you know, uh, mourning the future that I thought that I was gonna have."*
>
> **— PATIENT**
> EP. UTS_S4_PODCAST_026.DOCX

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A pervasive culture of silence surrounding women's health challenges, particularly during postpartum and midlife transitions, fosters profound feelings of isolation, shame, and the mistaken belief that one is alone in their struggle.

> *"So again, there, there's just so many ways to be. Isolated, like in, in postpartum. Um, and I, I'm very passionate about it, so that's why I wanna talk more about that aspect. Uh, even with breastfeeding, there's so much shame and judgment with it."*
>
> **— PATIENT**
> EP. UTS_S4_PODCAST_040.DOCX

When patients experience dismissal or a lack of answers from the healthcare system, they are compelled to take on the role of researcher and advocate for their own health. This often involves meticulously tracking symptoms, educating themselves on complex medical topics, and learning to persistently question providers and change doctors to overcome diagnostic delays and receive necessary care.

> *"After when you've been for three years and nobody's helping you. You need to do your own research. You need to advocate and learn and educate yourself."*
>
> **— PATIENT**
> EP. UTS_S4_PODCAST_028.DOCX

For clinicians and others in caregiving roles, the risk of burnout and vicarious trauma is immense, making the intentional practice of setting boundaries—such as unplugging from work and pouring back into oneself—a non-negotiable requirement for sustained effectiveness and personal wellbeing.

> *"I think the hallmark of boundaries in this work is. In order to be effective at what I do as a provider, I ha I cannot take it on. I have to purposefully unin mesh, right? I have to really be diligent and vigilant about. Unplugging and making sure that I can pour into me so that I can show up as the container that I want to be for my patients."*
>
> **— CLINICIAN**
> EP. UTS_S4_PODCAST_014.DOCX

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Sharing personal stories and listening to the lived experiences of other women serves as a powerful antidote to isolation, providing the validation, connection, and sense of shared humanity needed to navigate health challenges and systemic failures.

> *"healing is more about being seen. So what you need is find a place that you know you can feel validated. Connect with other women. You know, listen to other women's stories and don't stay stuck"*
>
> **— PATIENT**
> EP. UTS_S4_PODCAST_042.DOCX

In the face of both individual and collective trauma, actively building and leaning into a peer community is a primary protective factor, fostering a sense of belonging and shared strength that enables collective healing.

> *"collective trauma leads to collective healing. Which in a way I like that. I like that. I like that a lot. That's good. We look at each other here and we're all in it. And sometimes we forget, but most of the time when you slow down, you realize we're all in it together and we are gonna heal together. We got hurt together, we're gonna heal together."*
>
> **— ADVOCATE**
> EP. UTS_S4_PODCAST_016.DOCX

A growing number of professionals are actively rejecting the 'hustle culture' by establishing clear boundaries that place their personal and family well-being above work demands. This behavioral shift involves a conscious decision to prioritize self-care and family, with the understanding that a job is a part of life, not the entirety of one's identity or worth.

> *"we are number one, family is number two, and and unicorn is number three. And we all have to ensure that we are adjusting that way. And our team knows this as well. And anytime there is a moment we're like, no, you are first. Nope. Your family like, focus there. The job will get done."*
>
> **— PATIENT**
> EP. UTS_S4_PODCAST_023.DOCX

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METHODOLOGY

45

EPISODES ANALYZED

179

MOMENTS SEGMENTED

CONFIDENCE THRESHOLD

The report includes insights promoted to 'Medium' or 'High' confidence, based on multi-episode evidence and pattern coherence.

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